

Bold Funnel Conversion Audit — Senior / Medicare Persona

The Strategic Frame

The user you're acquiring is a **76-year-old on Medicare who clicked a Facebook ad promising pain/weight/energy help**. The ad sold *outcomes* ("feel better"); the funnel delivers *a medical appointment with a doctor they've never heard of, on Zoom, in 3 days*. The whole drop-off story is about the **gap between the ad's promise and the funnel's ask escalating too fast, with too little trust scaffolding** — and that gap got *worse* when you removed the Care Coordinator call, because the CC call was doing invisible work: framing, trust-building, expectation-setting, calendar-anchoring, and tech-readiness checks.

Every step below should be evaluated against one question: *Does this senior know what they just agreed to, why, and what happens next?*

Step-by-Step Drop-Off Analysis

Step 0 — Facebook Ad → Landing Page

What the user sees: Hero with "Lose weight. Reduce pain. Get your energy back. Covered by Medicare." + a form asking for First name / Last name / Email + "Check coverage" CTA.

Empathy read: The senior just clicked an ad about *feeling better*. They land on a page that immediately asks for PII before explaining what Bold is, who delivers care, or whether it's a doctor, coach, app, or class. The "Required" red label appears on First name *before they've typed anything* — this reads as error/scolding to seniors and creates anxiety.

Why 70% don't scroll:

- The hero form takes the full above-the-fold. **There is no visible signal that scrolling reveals more information** — no "How it works" peek, no scroll arrow, no preview imagery of providers.
- The form *is* the page. So they either fill it or bounce. There's no third option ("learn more first"), which is what a cautious senior wants.
- "Check coverage" is a vague CTA — it doesn't tell them what they're agreeing to. They don't know if they're starting an account, booking a visit, or just looking up benefits.

Drop-off reasons (ranked):

1. Outcome-to-mechanism gap: ad promises a benefit; page demands data. No "What is Bold?" in 1 sentence above the form.

2. Trust deficit: no provider photos, no "real doctors," no Medicare logo, no security badges visible above the fold.
3. Cognitive load: "Check coverage" is jargon to a senior — they think insurance verification, not appointment booking.
4. The premature "Required" red error state on the empty First name field signals "this site is going to be hard."
5. No phone number visible in the hero. Seniors trust phone numbers; the CC phone (424-577-5266) appears only after they commit.

Step 1 — Eligibility (State + DOB)

****What the user sees:**** "Help us confirm your coverage" with State dropdown and DOB field, plus consent language: ****By proceeding, you agree that Verified (Bold's service provider) and its vendors may receive your personal info..."**

****Empathy read:**** A senior just gave name + email, expecting a coverage check. Now they're suddenly facing a legal-looking consent involving a third party they've never heard of ("Verified"), and a DOB request. To a 76-year-old who reads scam-warnings from AARP, this screen pattern-matches to identity theft.

****Drop-off reasons:****

1. "Verified" and "its vendors" is uninvestigated third-party language with no explanation — creates fraud anxiety.
2. DOB requested without explaining **why** (eligibility = Medicare age) right next to the field. There's a "Why are we asking this?" link but it's an extra click, which seniors avoid.
3. State dropdown is alphabetical and long — fine on desktop, painful on mobile where most Facebook traffic lives.
4. No progress indicator. The user doesn't know if this is step 2 of 3 or step 2 of 12.

Step 2 — "You are covered!" Confirmation

****What the user sees:**** Green check, ****Based on your Medicare plan, your estimated out-of-pocket cost is \$0–\$35, and 86% of Bold patients pay \$0."**

****Empathy read:**** This is the funnel's emotional high point — but it also introduces a **new** cost anxiety (\$35) that wasn't on the landing page. A senior on a fixed income reading "\$0–\$35" will pause: "Which one is **me**?"

****Drop-off reasons:****

1. The \$0–\$35 range is the moment cost ambiguity is born. Without personalized resolution ("You will pay \$0"), some seniors abandon to call their Medicare rep first — and never come back.

2. "Next, let's match you with a provider" jumps directly into a clinical commitment with no soft step. The CC call used to be this soft step.
3. No mention of what kind of appointment is coming (Zoom vs phone, length, who, when).

Step 3 — Intake Question 1: Focus Area

****What the user sees:**** "Which area would you like to focus on?" — Weight management / Chronic pain / Energy & fatigue / General healthy aging.

****Empathy read:**** Fine in isolation. But seniors with multimorbidity ("I have arthritis *and* I'm tired *and* I want to lose weight") are forced to artificially single-pick. Some will abandon because "none of these fully describe me."

****Drop-off reasons:****

1. Single-select forces a false choice for a population whose defining feature is comorbidity.
2. No "I'm not sure" option — and "I'm not sure" is exactly when a Care Coordinator call would have rescued them.

Step 4 — Question 2: Medical Exclusions

****What the user sees:**** A 6-item checklist of disqualifying conditions (Stage 4/5 kidney disease, cancer treatment, eating disorder, alcohol/drug, weight loss surgery, unintentional weight loss).

****Empathy read:**** This is the funnel's biggest emotional landmine. A senior reading "Stage 4 or 5 kidney disease," "ongoing cancer treatment," and "eating disorder" feels they've walked into a serious medical clinic — *not* the cheerful "feel better, live bolder" brand the ad sold. The tone-shift is jarring and triggers "Is this for me? Am I sick enough? Am I too sick?"

****Drop-off reasons:****

1. Heavy clinical exclusions appear with no framing — looks like an ER intake form.
2. Anyone with one of these conditions hits a dead-end implicitly (we don't know what happens if they check a disqualifier — that branching deserves audit).
3. The list creates fear-anchoring right before the next ask, which depresses willingness to share.

Step 5 — Question 3: Open-text "One thing to discuss"

****What the user sees:**** Empty textarea with Continue + Skip.

****Empathy read:**** Seniors struggle most with open-text on mobile. Typing on a phone with arthritic fingers and presbyopia is the single highest-friction interaction in any senior funnel. Even with a Skip option, the cognitive load of *deciding whether to skip* is itself friction.

****Drop-off reasons:****

1. Open-text on mobile is the #1 mobile abandonment pattern for 65+.
2. Skip looks like a "wrong answer" — seniors are conditioned to fill every field.
3. No examples or chips to scaffold the answer.

Step 6 — Question 4: Open-text "Conditions/Concerns"

****What the user sees:**** *Another* open-text field with placeholder "e.g., I have high blood pressure and take medication every morning..."

****Empathy read:**** Back-to-back open-text fields. The senior is now wondering, "How much do they need? Will they read this? Did I already say this?" This is also where data-sensitivity anxiety re-emerges — they're typing medical history into a website they only met four screens ago.

****Drop-off reasons:****

1. Redundant feel with Q3 — both ask "tell us about you" in slightly different ways.
2. No reassurance about how this is used or who reads it before the appointment.
3. Skip again invites abandonment-by-decision-fatigue.

Step 7 — Provider Matching Loader

****What the user sees:**** "Matching you with the right Bold Care Provider" + three checkmarks (Licensed in CA / Focused on weight management / Covered by your insurance).

****Empathy read:**** A nice trust moment. Short, reassuring, *but* it's also the first place no progress indicator and no time estimate is given. Some seniors on slow connections may interpret the loader as broken.

****Drop-off reasons:****

1. No timer/estimate; some users tab away and never return.
2. Missed opportunity to use this dwell time to set expectations ("Next you'll meet Dr. X and pick a time. Your visit is by video. Need a phone visit? You can switch.").

Step 8 — Provider Reveal

****What the user sees:**** Dr. Mitul Desai, MD photo, "Internal medicine, 21+ years," "Frequently rebooked," "A great match for weight management," View profile / Continue.

****Empathy read:**** This is the funnel's **best** screen for trust — and also its biggest unintended consequence of the direct-booking switch. With the CC call, a human used to say: "I'm scheduling you with Dr. Desai — she's wonderful." Without it, the user has to do that emotional work alone, in 6 seconds, on a screen.

****Drop-off reasons:****

1. Only one provider is shown — no "Don't like this match? See others." For a senior who values choice in their doctor, single-option matching feels like being assigned, not choosing. (This is plausibly a major contributor to your post-switch schedule rate drop.)
2. No social proof at the provider level (reviews, # of patients seen, "Speaks Spanish," gender preference).
3. "View profile" is optional and many seniors won't click — they then book blind, which links directly to your ****no-show increase****: users who skip the profile have lower commitment.
4. "Frequently rebooked" badge is vague — seniors don't parse marketing language. "Patients return because they trust her" would resonate more.

Step 9 — Schedule

****What the user sees:**** "45-min Zoom video call with Dr. Mitul Desai." Soonest slots: Mon May 18 7AM/9AM/10AM PT, Tues 7AM, Wed 7AM. "Switch to phone call" link.

****Empathy read:**** This is the screen most directly tied to your ****no-show problem****. Three issues converge:

1. ****"45-min Zoom video call"***** is a **huge** commitment for a first-touch senior. The ad promised "feel better"; now the deal is 45 minutes on Zoom. Seniors who've never used Zoom may book it anyway out of politeness, then no-show because they couldn't get it working on the day.
2. ****Times shown are PT, very early (7AM)****, with limited variety. A senior in Pennsylvania (one of your eligible states) sees 7AM PT = 10AM ET — OK — but the **display** of "PT" alone confuses East Coast users.
3. ****"Switch to phone call"***** is a small text link, easy to miss. Phone calls have dramatically higher show rates for 65+ vs Zoom. You're hiding the highest-conversion modality.
4. ****No reminder of what the appointment is for****, *what to prepare, or what happens after*. The CC call used to do all of this verbally.

****Drop-off / no-show reasons:****

1. Zoom is the default; phone is buried. Seniors no-show Zoom appointments at 2–4× phone rates.
2. 45 minutes feels long for a "discovery" call — could be re-framed as "15-min welcome call, then a 30-min care plan visit."

3. No same-day or next-day slot creates a 3-day commitment gap — every hour between booking and visit increases no-show probability.
4. No calendar download (.ics), no SMS opt-in shown here, no "We'll text you a reminder" reassurance.
5. No mention of "We'll send a link" / "Here's a tech-check tool" — the seniors most likely to no-show are the ones least confident they can join.

Step 10 — HIPAA Consent (final gate)

****What the user sees:**** Wall of HIPAA legalese in a scrollable card with a single "I acknowledge" checkbox + Continue.

****Empathy read:**** Senior trust-anxiety peak. They've already given name, email, DOB, state, medical conditions, and chosen a time — **now** legal text appears. Some will abandon at this final gate because they want to "read it later" or "ask my daughter."

****Drop-off reasons:****

1. Legal wall-of-text shown **after** time selection, not before. Better to compress + show earlier or summarize in plain English with a "Read full notice" expand.
2. The checkbox + the giant scroll panel implies they should read it all. Seniors take that instruction seriously, freeze, and abandon.
3. No "Email me a copy" option — seniors love paper/email copies of legal docs.

Cross-Funnel Systemic Issues

Beyond per-step issues, four systemic problems explain the post-switch metrics:

****1. The Care Coordinator call was doing 7 invisible jobs.**** Removing it didn't just remove a step — it removed **expectation-setting, modality coaching (Zoom vs phone), cost resolution, provider warm-up, tech check, calendar anchoring, and human reassurance**. The current funnel replaces all 7 with zero scaffolding. Your schedule-rate and no-show metrics are reflecting that missing scaffolding.

****2. The funnel optimizes for completion, not commitment.**** Every screen pushes forward; nothing builds buy-in. A senior who **completes** booking without **internalizing** what they booked is exactly the profile that no-shows.

****3. Modality default is wrong for the population.**** Zoom-first for Medicare-age is a known anti-pattern. Phone should be equal or default, with Zoom as an "if you prefer video" option.

****4. There is no post-booking confirmation loop visible in the funnel.**** The biggest no-show lever in senior telehealth is **day-before-call human contact**. If you removed CC pre-booking, at minimum bring them back for a 2-minute confirmation call 24h before the appointment.

Prioritized Optimization Stack

Ranked by expected impact on ****scheduled rate**** and ****show rate****, weighted by implementation effort:

****Tier 1 — Highest ROI****

-  Restore a **lightweight** human touch for high-no-show segments: a 24-hour-pre-visit confirmation call/SMS by a Care Coordinator. Cheapest way to recover most of the no-show regression.
-  Make ****phone visit equal to Zoom**** on the schedule screen (two buttons, not a hidden link). Default to phone for users 75+.
-  Add a "Talk to a Care Coordinator first" option on the provider-match screen for users who hesitate (track hover/back-button signal). Recovers the segment that direct-booking lost.
- Rewrite the hero: lead with "Talk to a Medicare-covered doctor about pain, weight, and energy — from home." Add provider faces + "Most patients pay \$0" above the form. Add a secondary CTA "See how it works" that scrolls.

****Tier 2 — Funnel hygiene****

- Add a persistent progress bar ("Step 3 of 6") from eligibility onward.
- Replace the two open-text questions with chip-select + one optional textarea. Cut typing on mobile.
- Re-frame Q2 medical exclusions with warm copy: "A few quick safety questions so your doctor can plan your visit."
- Resolve cost ambiguity at the "You are covered" screen: show personalized \$ instead of "\$0-\$35."
- Show 2–3 provider options, not 1. Or show 1 with a "See other providers" link clearly visible.
- Move HIPAA acknowledgement **before** time selection, summarized in plain English with optional full text.

****Tier 3 — No-show specific****

- After booking, route every patient through a 60-second tech check (camera/mic test for Zoom; verify callback number for phone).
- Auto-send .ics calendar invite + SMS opt-in on confirmation screen.
- Day-of: text 1 hour before with "Tap here to join — need help? Reply HELP."
- Re-frame visit duration: "15-min intro + 30-min care plan" rather than monolithic "45-min Zoom call."

****Tier 4 — Trust and tone****

- Replace "Verified (Bold's service provider) and its vendors" with plain-language explanation + lock icon.
- Add Medicare and UnitedHealthcare logos near the eligibility step.
- Replace "Frequently rebooked" badge with concrete proof ("Seen by 1,200+ patients, 4.9★").
- Visible phone number on *every* funnel step, not just after commitment.

What I'd A/B Test First (next 30 days)

1. Phone-equal-to-Zoom on schedule screen → expected lift: no-show ↓.
2. "Talk to a Care Coordinator first" escape-hatch on provider-match screen → expected lift: schedule rate ↑ (recovers the segment direct-booking losses).
3. 24-hour pre-visit Care Coordinator confirmation outbound → expected lift: show rate ↑ (the single biggest lever).
4. Hero rewrite with "How it works" secondary CTA → expected lift: hero scroll % ↑ from 30% toward 50%+.

If you want, I can mock up rewritten copy for the hero, the eligibility consent, and the schedule screen, or draft the Care Coordinator confirmation script for the 24-hour pre-visit call.